CMF · intravenous (IV) · 28-day schedule

All three medicines are given intravenously on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Mouth

Methotrexate / fluorouracil

Ulcers or difficulty drinking need prompt advice.

2 Bowel

Methotrexate / fluorouracil

Report diarrhoea, vomiting or poor intake.

3 Blood / infection

All three medicines

Fever, shivering or sudden illness: call immediately.

4 Kidneys / fluid

Methotrexate

Kidney function affects clearance; report reduced urine.

5 Bladder

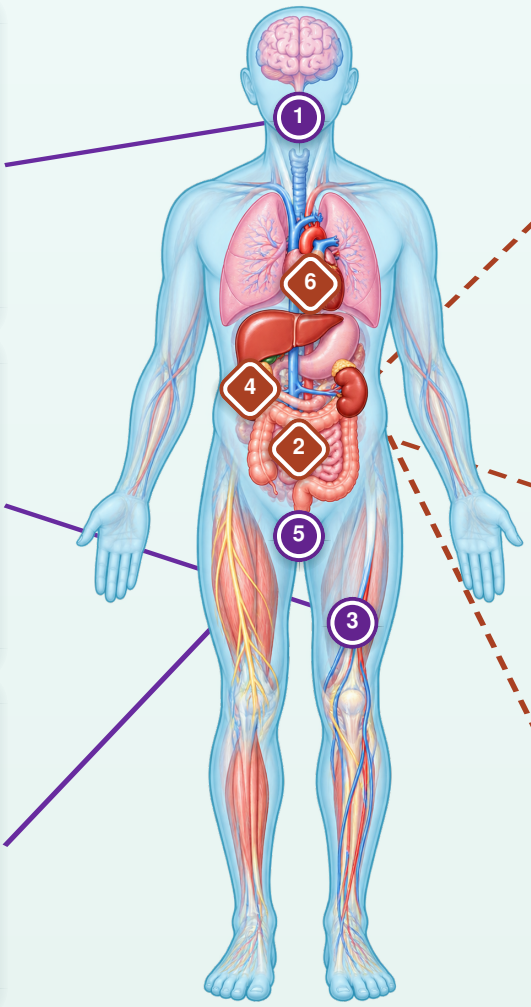
Cyclophosphamide

Report painful urination or blood in urine.

6 Chest / breathing

Fluorouracil / methotrexate

New chest pain or breathlessness needs urgent assessment.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Medicine-specific effects are highlighted; this is not a complete list. New or persistent symptoms still need review after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Cyclophosphamide

- Low counts and infection
- Nausea, vomiting and hair loss
- Bladder irritation / bleeding
- Fertility effects and menopause
- Rare later blood cancers

Methotrexate

- Low blood counts and serious infection
- Mouth ulcers and diarrhoea
- Liver injury and prolonged exposure with kidney impairment
- Uncommon serious lung inflammation

Fluorouracil (5-FU)

- Low blood counts and infection
- Diarrhoea and mouth ulcers
- Rare serious heart symptoms
- Severe toxicity with dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) deficiency

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Anti-sickness plan

Follow the prevention and rescue instructions prescribed for this schedule. Call if vomiting prevents drinking.

Infection and bleeding

Know the local urgent-contact number. Fever, shivering, sudden illness or unusual bleeding needs prompt advice even between visits.

Medicine review

Show the oncology pharmacist all prescribed, over-the-counter and herbal medicines. Ask before starting anything new.

Hydration and symptoms

Agree fluid intake with the team, especially with heart or kidney fluid restrictions. Report reduced urine, swelling, mouth ulcers or diarrhoea.

DPD result and methotrexate clearance

Confirm DPD testing before the first fluorouracil dose. Kidney impairment can affect interpretation of uracil-based testing; ask the oncology team to review the result and testing method. Tell the team about fluid around the lungs or in the abdomen, which can prolong methotrexate exposure.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00378/

Sources and review status are available online. Follow your own prescription and hospital instructions.